

RANZCP MEQ Examination Pilot Monday 24 November 2025

=====

Modified Essay Questions (MEQs): There are five (5) MEQs worth a combined total of 125 marks.

Each MEQ is worth between 10 and 39 marks.

=====

*** Modified Essay 1 has a total of 39 marks with 4 parts. ***

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist working on a Consultation Liaison team in a metropolitan hospital. Adil is a 32-year-old single Afghan refugee with a diagnosis of schizophrenia and intravenous heroin misuse. He is under a community mental health team and is prescribed depot antipsychotic medication. Adil initially agreed to be admitted to the medical ward for suspected infective endocarditis. However, he has since refused to have any tests done, and has tried to leave the medical ward. You have been asked to assess Adil as his medical team is concerned about his treatment refusal.

Question 1.1

Outline (list and justify) the factors that might be contributing to Adil's treatment refusal.

Please note: a list without any justification will not be awarded any marks.

(10/39 marks)

Marking criteria:

A. Patient factors (4 marks):

- Language barriers: understanding of his current condition; is there a need for an interpreter?
- Cognitive impairment related to schizophrenia or substance dependence.
- Cultural factors: non-western explanatory models of illness, including spiritual explanation for his illness.
- Health literacy.
- Insight: how informed is he about his medical condition?
- Fears of implications of having another illness such as the uncertainty of his residency status.
- Trauma-related triggers – being kept in hospital; mistrust of authorities (treating team); personality.
- External commitments or obligations, including financial concerns with being in hospital.

B. Illness factors (may or may not impair Adil's capacity for consenting) (3 marks):

- Depression – reluctance, hopelessness for prospects for treatment.
- Psychosis – inadequately treated; substance induced.
- Delirium – infection, etc.
- Physical illness (non-delirium causes) e.g. malnourishment, pain, drug intoxication/withdrawal/craving.
- Cognitive impairment secondary to schizophrenia.
- Opiate withdrawal (acute symptoms including agitation, restlessness and confusion given history of intravenous heroin use).

C. Systems factors (3 marks):

- Paternalistic approach – medical teams may present as paternalistic in their approach to patient care (consider person-centred care, collaborative approach).
- Family factors – social isolation; visiting policies.
- Cultural awareness with respect to the treating team lacking awareness of culturally safe care.
- Stigma about patients with mental illness or those who abuse substances; blame attribution. Refugee stigma.
- Negative countertransference.

RANZCP MEQ Examination Pilot Monday 24 November 2025

The information that is presented in italics in this question is a repetition of the earlier sections of the case vignette.

**** Modified Essay 1 has a total of 39 marks with 4 parts. ****

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist working on a Consultation Liaison team in a metropolitan hospital. Adil is a 32-year-old single Afghan refugee with a diagnosis of schizophrenia and intravenous heroin misuse. He is under a community mental health team and is prescribed depot antipsychotic medication. Adil initially agreed to be admitted to the medical ward for suspected infective endocarditis. However, he has since refused to have any tests done, and has tried to leave the medical ward. You have been asked to assess Adil as his medical team is concerned about his treatment refusal.

Adil consents to stay on the ward and to the recommended cardiac investigations. A week later, he is prescribed methadone and intravenous antibiotics. The medical team have asked you to review Adil again as he has appeared sad, unusually quiet and pre-occupied to the treating team. He has also told the nursing staff that he has been having poor sleep for more than three months.

Question 1.2

Outline (list and justify) the points you would consider in your review of Adil's current presentation.

Please note: a list without any justification will not be awarded any marks.

(12/39 marks)

Marking criteria:

A. Mood assessment (3 marks):

- Differentiating a major depressive episode from a substance-related mood episode (e.g. methadone undertreatment or overtreatment).
- Past manic or hypomanic symptoms.
- Personality factors.
- Affective episode with psychotic features.
- Medication-induced mood disorder.
- Adjustment disorder with depressed mood (in response to a stressor but not meeting criteria for an MOE).
- Non-pathological periods of sadness (short duration, few associated symptoms, lack of significant functional impairment or distress).

B. Sleep Assessment (3 marks):

- Clarify nature of sleep difficulty (initial insomnia, middle insomnia, early morning awakening). Identifiable factors interfering with falling asleep (e.g. anxious ruminations, racing thoughts, pain etc.). Nightmares. Restless legs. Waking up unrefreshed. Snoring.
- Consider influence of methadone and medications on sleep; drug withdrawal (methadone undertreatment); sleep disturbance as a symptom of depression or anxiety; lifestyle factors (poor sleep hygiene) affecting sleep; circadian rhythm disturbance; breathing-related sleep disorders (e.g. sleep apnoea).

C. Physical health assessment (1 mark):

- Liaise with medical team regarding the progress of his endocarditis treatment; prognosis; need for follow up appointments or investigations; malnourishment.

D. Trauma and stress-related disorder assessment (1 mark):

- Consider PTSD; complex PTSD (complex trauma); adjustment disorder; recent grief and loss; adjustment disorder; dissociative disorders (depersonalisation/derealisation disorder).

E. Other psycho-social stressors assessment (1 mark):

- Poverty, unstable accommodation, food insecurity, sense of safety, connectedness to community and country, re-traumatisation by events in his home country (vicarious trauma).

F. Co-morbid substance abuse (1 mark):

- Other substances used not declared, use of non-prescribed medications, over the counter medications, change in pattern of use, etc.

G. Exacerbation of psychotic sx secondary to the stress of an infective illness, hospitalisation, etc. (1 mark):

- Comprehensive review of symptoms of psychosis including hallucinations, delusions and abnormal behaviours.

- Establish his baseline mental state and whether current presentation is different from that.

H. Delirium (1 mark), due to resulting acute illness (endocarditis) and possibly due to complications such as venous thrombosis and/or cerebral vascular events due to endocarditis.

RANZCP MEQ Examination Pilot Monday 24 November 2025

The information that is presented in italics in this question is a repetition of the earlier sections of the case vignette.

**** Modified Essay 1 has a total of 39 marks with 4 parts. ****

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist working on a Consultation Liaison team in a metropolitan hospital. Adil is a 32-year-old single Afghan refugee with a diagnosis of schizophrenia and intravenous heroin misuse. He is under a community mental health team and is prescribed depot antipsychotic medication. Adil initially agreed to be admitted to the medical ward for suspected infective endocarditis. However, he has since refused to have any tests done, and has tried to leave the medical ward. You have been asked to assess Adil as his medical team is concerned about his treatment refusal.

Adil consents to stay on the ward and to the recommended cardiac investigations. A week later, he is prescribed methadone and intravenous antibiotics. The medical team have asked you to review Adil again as he has appeared sad, unusually quiet and pre-occupied to the treating team. He has also told the nursing staff that he has been having poor sleep for more than three months.

Following your assessment of Adil, you form an opinion that Adil is moderately-to-severely depressed.

Question 1.3

Describe (list and explain) your management at this point.

Please note: a list without any explanation will not be awarded any marks.

(10/39 marks)

Marking criteria:

A. Location of treatment (1 mark):

- Adil will likely still be in the ward for a little while so treatment can commence in the ward, with a plan to provide a handover to the outpatient team. This would include collaborative discharge planning in advance of the discharge.

B. Biological (2 marks):

- Consider antidepressants for MDD; explore if previously experienced depression and how it was treated; note potential for interactions like serotonin syndrome (use caution with SSRIs; consider mirtazapine); sedatives – potential for interactions; QTc prolongation – caution with quetiapine.
- Melatonin for sleep onset.
- Optimise methadone. Consider buprenorphine–naloxone combination.

C. Social (1 mark):

- Involvement of the social worker (address migration issues and advocate for him where relevant, financial and housing support and advocacy, community supports linkages); explore discharge disposition.
- Liaison with Adil regarding available social supports (family or friends) to inform discharge planning.

D. Cultural (1 mark):

- Involvement of a trans-cultural worker to explore culturally safe supports and interventions, including spirituality.

E. Liaison with treating team (1 mark):

- Education/support including Cultural awareness training, if indicated.
- Assessment and management of risk (suicide, self-harm, medical, absconding).
- Management of behaviours of concern.

F. Psychological interventions (start in the ward) (4 marks):

- Recovery based approach:
 - Engage consumer consultant, peer support worker, or other support person.
 - Ensure there is a care plan in place.
 - Encourage self-agency.
 - Empowering Adil to identify his strengths and set his recovery goals.
 - Ensure Adil is aware of his rights.
- Psychoeducation regarding major depressive episode, its prognosis, journey and share the formulation.
- Consider a referral to a (health) psychology service if available to work with Adil around adjustment with serious physical illness.
- Consider supportive psychotherapy approach in hospital to assist with depressed mood.

- Consider a behavioural activation plan if Adil will stay in hospital for a prolonged period for endocarditis treatment in order to address low mood.

RANZCP MEQ Examination Pilot Monday 24 November 2025

The information that is presented in italics in this question is a repetition of the earlier sections of the case vignette.

**** Modified Essay 1 has a total of 39 marks with 4 parts. ****

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist working on a Consultation Liaison team in a metropolitan hospital. Adil is a 32-year-old single Afghan refugee with a diagnosis of schizophrenia and intravenous heroin misuse. He is under a community mental health team and is prescribed depot antipsychotic medication. Adil initially agreed to be admitted to the medical ward for suspected infective endocarditis. However, he has since refused to have any tests done, and has tried to leave the medical ward. You have been asked to assess Adil as his medical team is concerned about his treatment refusal.

Adil consents to stay on the ward and to the recommended cardiac investigations. A week later, he is prescribed methadone and intravenous antibiotics. The medical team have asked you to review Adil again as he has appeared sad, unusually quiet and pre-occupied to the treating team. He has also told the nursing staff that he has been having poor sleep for more than three months.

Following your assessment of Adil, you form an opinion that Adil is moderately-to-severely depressed.

Three weeks later, Adil is ready to be discharged. You conduct a discharge assessment before he leaves the medical ward. He has been referred to a community opioid replacement programme and currently receives 60mg of methadone daily.

Question 1.4

Outline (list and justify) the key points you would cover in your discharge planning meeting with Adil.

Please note: a list without any justification will not be awarded any marks.

(7/39 marks)

Marking criteria:

A. Points to cover (3 marks):

- Create a safety plan, early warning signs of mental state deterioration, crisis and emergency contact. (Note to markers: candidates may/should also state something like, "conduct a risk review").
- Advise follow-up and facilitate appointments for CMH team, addictions services, GP. If possible, invite community team case manager to the discharge planning meeting.
- Ensure treatment is continued e.g. pharmacy for opioid replacement, next depot, script for medication when discharged.
- Psychoeducation around medications, treatment plan – education about the indications for his prescribed medication, adverse effects, recommendation for the use of medication blister (Webster) packs to aid compliance. Explore potential for Adil to seek out traditional, healing methods as this could have implications for his treatment (interaction, non-adherence).
- Advise re: risks of substance use and methadone: risk of accidental overdose (if he uses the usual amount of heroin), and death.
- Harm minimisation including naloxone; needle exchange; safe injection practices.
- Along with the medical team, ensure Adil understands the importance of adhering to treatment for endocarditis and the risks of premature cessation.

B. Linking in with a GP for follow-up of holistic care of physical and mental health problems (1 mark)

C. Social (2 marks):

- Importance of stable living arrangements; access to support services such as free meals, shelters, self-care.
- Family/social support engagement where possible.
- Migration issues including linking with cultural supports in the community.
- Encourage options for interpreter if required.
- Encourage self-agency, Recovery paradigm.

D. Recommend trauma-informed psychological therapy in the medium-to-long term (1 mark)

RANZCP MEQ Examination Pilot Monday 24 November 2025

*** Modified Essay 2 has a total of 32 marks with 3 parts. ***

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist working in a regional mental health centre. A mental health nurse practitioner calls you after hours to discuss Daniel, a 12-year-old indigenous boy, who was brought by police to the Emergency Department after he damaged property at his group home and threatened to hurt himself. He is currently settled and playing a card game with the carer. He sees a therapist at a local child and adolescent mental health service and is prescribed psychotropic medication.

Daniel's care was assumed by statutory child protective services at 5 years of age, when his mother was murdered by his father after years of family violence. He moved to his tenth group home a week ago.

Question 2.1

Describe (list and explain) the various factors likely to have contributed to the incident that resulted in Daniel's hospital presentation.

Please note: a list without any explanation will not be awarded any marks.

(14/32 marks)

Marking criteria:

A. Current conditions and treatments (3 marks):

- Mental illness – relapse of psychiatric illness or exacerbation of a current mental disorder, including developmental trauma, PTSD.
- May have increased predisposition due to stress.
- Medications may have lapsed, maladministration of medication, medication interactions.
- Physical comorbidity:
 - Brain damage, fetal alcohol syndrome, intrauterine issues.
 - Possible developmental insult e.g. physical injury, exposure to alcohol and other drugs – impact on cognitive capacity and capacity for emotional regulation.

B. Daniel's current life experience (3 marks):

- Limited trust and adjustment – difficult to establish in a new setting with a traumatised child.
- Unknown stressors/losses – associated with dislocation, concern about friends left behind and fear of further abandonment.
- Behaviour as a means of communicating his distress.
- Disruption to schooling and extra-curricular activities: possible change of schools, loss of friends/peer support, etc. Staff and group home.
- Staff attitudes, experience/inexperience and behaviours: for various reasons (e.g. fear of Daniel, unhelpful judgements).

C. Development and attachment (6 marks):

- Understand presentation in context of developmental stage e.g. Freud's latency period or Erikson's industry vs inferiority– current circumstances limit access to a safe or predictable environment in which to develop a sense of mastery.
- Limited social connection – lack of a stable friendship circle and disruption to previous friendships impact the development of social and relational skills.
- History/current trauma, abuse, neglect – impact on attachment style and attitude to caregivers.
- Impacts significantly on affect regulation, sense of self and interpersonal relationships and trust.
- Attachment style (e.g. anxious ambivalent, anxious avoidant with intrusions of negative affect, disorganised). Behaviours reflect strategies to feel safe.
- Lack of stability – recent disruption in out of home care may contribute to a sense of impermanence and affect trust.
- Inconsistent education – limited socialisation, disrupted learning.

D. Cultural and broader social factors (2 marks):

- Indigenous background – part of a marginalised group with major intergenerational trauma.
- Daniel's assumption of care can be seen as another example of "stolen generation" (Australian context).
- A lack of cultural sensitivity within the group home, including institutional or perceived racism.
- Intergenerational patterns – genetic vulnerability, intergenerational transmission of trauma.

RANZCP MEQ Examination Pilot Monday 24 November 2025

The information that is presented in italics in this question is a repetition of the earlier sections of the case vignette.

**** Modified Essay 2 has a total of 32 marks with 3 parts. ****

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist working in a regional mental health centre. A mental health nurse practitioner calls you after hours to discuss Daniel, a 12-year-old indigenous boy, who was brought by police to the Emergency Department after he damaged property at his group home and threatened to hurt himself. He is currently settled and playing a card game with the carer. He sees a therapist at a local child and adolescent mental health service and is prescribed psychotropic medication.

Daniel's care was assumed by statutory child protective services at 5 years of age, when his mother was murdered by his father after years of family violence. He moved to his tenth group home a week ago.

You learn that Daniel is prescribed the following medications: olanzapine, extended-release methylphenidate, sertraline, sodium valproate and, melatonin.

The nurse practitioner advises that Daniel's carer has stated he has childhood bipolar disorder and has asked for advice about managing his behaviour in the group home. Daniel has not responded to the long-standing behaviour management plan where Daniel is rewarded with screen time for being "good" and loses privileges when he has been "bad".

Question 2.2

Describe (list and explain) the reasons Daniel may not respond to this behaviour plan.

Please note: a list without any explanation will not be awarded any marks.

(11/39 marks)

Marking criteria:

A. Behaviour management plan factors (5 marks):

- Lack of an individualised behaviour support plan.
- The plan is not trauma-informed. The focus on behaviour, with its positive and negative consequence regimen, is crude and potentially punitive: does not formulate difficult behaviour in terms of attachment need/response to threat and trauma exposure.
- Relationships as basis of plan – plan may not identify or support quality of relationships with (new) carers.
- Length and language of plan – the plan may be too long or technical to be usable. "Good behaviour" and "bad behaviour" is unhelpful, for carers and for Daniel.
- Associated training of staff – staff may have had limited training in the use of the plan, especially noting Daniel has only been at the group home for a week.
- Plan detail may not have been contextualised for the new group home.
- There may be disagreement between staff or services about the focus and content of the plan.

B. Individual factors (4 marks):

Daniel's capacity to understand the plan:

- Daniel's cognitive capacity/comprehension/literacy may limit his ability to comprehend the plan, especially if stressed by a new environment etc.
- Medication e.g. valproate, olanzapine may affect comprehension or levels of alertness.

Daniel's experience of the world:

- Daniel's mental state may be contributing to heightened agitation (e.g. ADHD, mania, distress related to trauma and change, dose of stimulant/SSRI or interaction) or reduced attention (e.g. distractibility/hypervigilance, manifestation of depression, iatrogenic effect of olanzapine, ADHD) leading to difficulty complying with the plan.
- Possible use of alcohol and drugs.
- Daniel's need for safety and predictability may be expressed through his behaviour and the plan may not reflect this.

C. Systems, organisational, other biopsychosocial, cultural & spiritual factors (2 marks):

- The plan may not be sufficient to contain the distress and anxiety in the system, which may be associated with risk concerns, staff level of training/experience/support, or staff countertransference reactions.
- Intergenerational factors (genetic and non-genetic) e.g. distrust of the system in an indigenous family, impact of stolen generation (in the Australian context) and institutional discrimination – plan may be experienced as another form of paternalism/oppression.
- Lack of cultural sensitivity/awareness within the care team, group home including institutional or perceived racism.

RANZCP MEQ Examination Pilot Monday 24 November 2025

The information that is presented in italics in this question is a repetition of the earlier sections of the case vignette.

**** Modified Essay 2 has a total of 32 marks with 3 parts. ****

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist working in a regional mental health centre. A mental health nurse practitioner calls you after hours to discuss Daniel, a 12-year-old indigenous boy, who was brought by police to the Emergency Department after he damaged property at his group home and threatened to hurt himself. He is currently settled and playing a card game with the carer. He sees a therapist at a local child and adolescent mental health service and is prescribed psychotropic medication.

Daniel's care was assumed by statutory child protective services at 5 years of age, when his mother was murdered by his father after years of family violence. He moved to his tenth group home a week ago.

You learn that Daniel is prescribed the following medications: olanzapine, extended-release methylphenidate, sertraline, sodium valproate and, melatonin.

The nurse practitioner advises that Daniel's carer has stated he has childhood bipolar disorder and has asked for advice about managing his behaviour in the group home. Daniel has not responded to the long-standing behaviour management plan where Daniel is rewarded with screen time for being "good" and loses privileges when he has been "bad".

Question 2.3

Discuss (list and debate) the issues you would consider in relation to Daniel's diagnosis of childhood bipolar disorder.

Please note: a list with no debate will not be awarded any marks.

(7/39 marks)

Marking criteria:

A. Clinical presentation (4 marks):

Answers that consider possible affirming or contradicting symptoms but maintain a critical perspective and include formulation, contribute to these marks.

- Symptoms and mental state: current presentation does not suggest mania (e.g. able to settle), but this does not preclude the diagnosis. Diagnosis, if made, would depend on detailed description of manic behaviour and specific symptoms including sustained elevation/irritability, disinhibition, sleep disruption, potentially discrete depressive episodes and a formulation that considered biological, psychological, social and cultural factors.
- Alternative possible explanations include: other diagnostic possibilities e.g. disruptive behaviour syndromes, anxiety disorders, dissociative syndromes, substance use, and behaviours understood from a trauma and attachment style perspective.
- Awareness of the differential of psychotic symptoms and/or dissociative symptoms in patients exposed to severe interpersonal and developmental trauma.
- Treatment response: consider response to interventions (psychosocial and pharmaceutical) and recognise that changes to clinical picture may have multiple contributing factors.
- Family history may suggest bipolar disorder.
- Exercise caution especially noting history of family and systemic trauma, marginalisation, and potential to understand behaviour through attachment and trauma, not just biology.
- Consider any potential role of alcohol or drugs on clinical presentation.

B. Sources of information (2 marks):

Answers that consider the range of information sources, the differences in perspective and in what they contribute, and the need to be circumspect score marks in this section.

- Consider Daniel's account, carers' accounts (including past carers), child protection services' account, and clinical reports.
- Specifically explore clinical reasoning utilised when conferring the diagnosis.
- Recognise that different sources will see different aspects of the complex presentation and this may be a limitation.

C. Debate in the literature on diagnosis (1 mark):

- Data interpretation.
- Validity of diagnosis.
- Neurodevelopmental impact.
- Does not necessarily predict adult bipolar disorder.

RANZCP MEQ Examination Pilot Monday 24 November 2025

*** Modified Essay 3 has a total of 25 marks with 5 parts. ***

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist in an acute psychiatric inpatient unit attached to a teaching hospital. You are considering prescribing an antipsychotic medication for a young female science student, Nicolette, who has presented with a first episode psychosis. You requested a baseline ECG and it has come back "unremarkable".

Question 3.1

List the elements of an ECG trace recording that you would review.

(5/25 marks)

Marking criteria:

A. Systematic Interpretation of an ECG tracing (5 mark):

- Heart rate, rhythm (sinus, AF), and axis.
- Intervals/segments: PR interval, QRS duration, QT interval (start of q-wave to the end of the t-wave), ST Segment.
- QT interval is the length of time required for the heart to repolarise following the onset of depolarisation.
- QTc is QT corrected for heart rate.
- Conduction analysis: normal, BBB etc.
- Waveform description (i.e. T wave morphology, etc.).
- Compare with previous ECGs if available.
- Correlate the ECG findings with clinical presentation/context in interpreting the ECG.

Note to marker: Candidate must state rhythm, rate and axis to score 1 mark.

RANZCP MEQ Examination Pilot Monday 24 November 2025

The information that is presented in italics in this question is a repetition of the earlier sections of the case vignette.

**** Modified Essay 3 has a total of 25 marks with 5 parts. ****

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist in an acute psychiatric inpatient unit attached to a teaching hospital. You are considering prescribing an antipsychotic medication for a young female science student, Nicolette, who has presented with a first episode psychosis. You requested a baseline ECG and it has come back "unremarkable".

Question 3.2

Describe (list and explain) the risks of prolonged QTc interval.

Please note: a list without any explanation will not be awarded any marks.

(4/25 marks)

Marking criteria:

A. Risks of prolonged QTc interval (4 marks):

- State of increased vulnerability to malignant ventricular arrhythmias, syncope, and sudden death.
- Duration of QTc greater than 440ms (0.44s) for males or 470ms (0.47s) for females increases risk (suggested by limited evidence).
- Duration of QTc greater than 500ms (0.50s) is linked to an increased risk of arrhythmias (stronger evidence), Torsade de pointes (QTc intervals >650ms may be more likely than not to induce torsades).
- Overall risk is related to dose, plasma concentration and polypharmacy.
- When the dose is increased or adjunctive medications are being considered, QTc interval is likely to increase.

RANZCP MEQ Examination Pilot Monday 24 November 2025

The information that is presented in italics in this question is a repetition of the earlier sections of the case vignette.

**** Modified Essay 3 has a total of 25 marks with 5 parts. ****

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist in an acute psychiatric inpatient unit attached to a teaching hospital. You are considering prescribing an antipsychotic medication for a young female science student, Nicolette, who has presented with a first episode psychosis. You requested a baseline ECG and it has come back "unremarkable".

Question 3.3

List physical health factors which increase the risk of QTc prolongation in patients with psychiatric conditions

(3/25 marks)

Marking criteria:

A. Physical health factors: (3 marks):

- Risk may be compounded by lifestyle factors, and in patients with other comorbidities often associated with chronic psychiatric conditions such as metabolic syndrome, obesity, smoking-related health effects, impaired GTT.

Medical conditions:

QTc prolongation can be caused by a range of medical conditions.

- Increased risk with female gender, stress/shock, anorexia nervosa, extremes of age.
- Congenital conditions – Wolff–Parkinson–White syndrome, etc.
- Cardiac conditions – left ventricular hypertrophy, heart failure, myocardial infarction.
- Electrolyte imbalance – hypokalemia, hypomagnesaemia, hypocalcaemia.

RANZCP MEQ Examination Pilot Monday 24 November 2025

The information that is presented in italics in this question is a repetition of the earlier sections of the case vignette.

**** Modified Essay 3 has a total of 25 marks with 5 parts. ****

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist in an acute psychiatric inpatient unit attached to a teaching hospital. You are considering prescribing an antipsychotic medication for a young female science student, Nicolette, who has presented with a first episode psychosis. You requested a baseline ECG and it has come back "unremarkable".

Question 3.4

List medications which increase the risk of QTc prolongation.

(5/25 marks)

Marking criteria:

A. Psychotropic medications known to cause prolonged QTc in high doses (3 marks):

Antipsychotic agents:

- High effect: pimozide; drug combinations; excessive dosing; any intravenous antipsychotic.
- Moderate effect: amisulpiride, chlorpromazine, haloperidol, quetiapine, ziprasidone.
- Low effect: aripiprazole, asenapine, clozapine, Olanzapine, paliperidone, risperidone, sulpiride.
- No effect: lurasidone, brexpiprazole, cariprazine.

Antidepressants:

- SSRIs – citalopram, escitalopram, fluoxetine, sertraline, fluvoxamine, paroxetine.
- Venlafaxine.
- Tricyclic antidepressants.

Mood stabilisers:

- Lithium.

B. Commonly used non-psychotropic medications (2 marks):

- Antibiotics – azithromycin, clarithromycin, erythromycin, roxithromycin, metronidazole (with alcohol), moxifloxan.
- Antifungals – fluconazole, (in cirrhosis) ketoconazole.
- Antivirals – nelfinavir.
- Antimalarials – chloroquine, mefloquine
- Methadone.
- Anti-arrhythmic medications amiodarone.
- Immunosuppressants – cyclosporin.

RANZCP MEQ Examination Pilot Monday 24 November 2025

The information that is presented in italics in this question is a repetition of the earlier sections of the case vignette.

**** Modified Essay 3 has a total of 25 marks with 5 parts. ****

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist in an acute psychiatric inpatient unit attached to a teaching hospital. You are considering prescribing an antipsychotic medication for a young female science student, Nicolette, who has presented with a first episode psychosis. You requested a baseline ECG and it has come back "unremarkable".

Nicolette is prescribed an antipsychotic. A week later, her repeat ECG reveals a QTc interval of 480ms.

Question 3.5

Outline (list and justify) your approach to this finding.

Please note: a list without any justification will not be awarded any marks.

(8/25 marks)

Marking criteria:

A. (1 mark):

- Advise patient (and/or family) of ECG changes, of minimising risk etc. We have a duty of care and an ethical obligation for full disclosure (Open disclosure/Statutory Duty of Candour).
- Include Nicolette in the discussion.
- Provide psychoeducation about QTc prolongation (may need to be repeated at intervals depending on her mental state).

B. (1 mark):

- Repeat the ECG as the tracing could have been affected by time of day, food and alcohol intake (e.g. whilst on leave from the ward).

C. (1 mark):

- Review past medical history including prior cardiac history.
- Review past ECGs (including baseline) and compare, as it may not be a medication effect.

D. (1 mark):

- Review current prescription – including the use of other medications, such as an antidepressant (like some SSRIs) which may inhibit the metabolism of the antipsychotic.

E. (1 mark):

- Review benefit vs risk, especially if there is no comorbidity or other risk factors.

F. (1 mark):

- Consider reduction in dose or change of medication to an anti psychotic less likely to cause Qtc changes (female, QTc >470ms).

G. (0 mark):

- Consider a cardiology opinion if the repeat ECG reveals prolongation of QTc. (QTc > 470 ms but <500 ms) (may not be necessary as Qtc <500ms).

H. (1 mark):

- Consider use of other medication (within scope) e.g. beta blocker, magnesium supplementation.

Note to marker: Most psychiatrists would not have this within their scope of practice.

I. (1 mark):

- Routine ECG monitoring for QTc prolongation and abnormal T wave morphology.

- Frequency of ECG monitoring – at least annually, or when medication dose is increased, or when add-on medications are being considered.

J. (0 mark):

- Review and investigate possible electrolyte abnormalities e.g. magnesium, potassium.

RANZCP MEQ Examination Pilot Monday 24 November 2025

*** Modified Essay 4 has a total of 10 marks with 2 parts. ***

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist working in the community mental health clinic of a metropolitan hospital. You have been asked to review Julia, a 52-year-old woman who was referred by her general practitioner (GP). Julia is complaining of tearfulness, an inability to sleep, and excessive anxiety. Julia recently considered stopping her prescribed sertraline 100mg daily, because of weight gain and fatigue. The GP recommended a gradual reduction over a period of four weeks. Julia's current symptoms occurred in week 3 of a reducing regime, after she was reduced to a daily dose of 25mg. Her GP has diagnosed her with relapse of anxiety disorder but would like to get a second opinion.

Question 4.1

Outline (list and justify) key considerations to distinguish relapse of an anxiety disorder from antidepressant withdrawal.

Please note: a list without any justification will not be awarded any marks.

(6/10 marks)

Marking criteria:

A. Patient factors (2 marks):

- Past history of anxiety.
- Psychological factors: medication change is usually a time for apprehension and anxiety about relapse.
- Emotions compared to cognition offer little in the way of discriminative power. In practice, somatic symptoms along with unusual neurocognitive symptoms are most helpful.
- F.I.N.I.S.H. (Flu-like symptoms, insomnia, nausea, imbalance, sensory disturbances, and hyperarousal).

B. Medication factors (3 marks):

- Type of antidepressant:
 - Half-life (fluoxetine by virtue of its long half-life is inherently less likely to cause withdrawal).
 - Selective vs broader-spectrum antidepressants (TCAs and MAOIs are more likely to cause withdrawal). Some antidepressants are known to pose similar problems despite being more selective (sertraline and paroxetine).
 - For agents with marked differences in peak-trough plasma levels, an increase in the frequency of administration can reduce these fluctuations and the likelihood of inadvertent withdrawal effects.
- Temporal relationship between withdrawal and relapse symptoms: "1-2 rule": withdrawal symptoms appear 1-2 days after commencement of withdrawal, reaching peak severity at 1-2 weeks and eventually remitting within 1-2 months. Symptoms of relapse have a slower onset and once established, they remain undiminished.
- Use of DESS (Discontinuation – Emergent Signs and Symptoms) checklist (Rosenbaum 1998).
- Newer antidepressants: These have a much faster onset of action with potential side-effects that overlap with antidepressant withdrawal (e.g. esketamine).

C. Rate of withdrawal of antidepressant (1 mark):

- Faster withdrawal is more likely to lead to withdrawal symptoms and increased intensity.
- Style of tapering.

Co-occurrence:

- Withdrawal and relapse symptoms are not mutually exclusive (co-occurrence is perhaps common).

RANZCP MEQ Examination Pilot Monday 24 November 2025

The information that is presented in italics in this question is a repetition of the earlier sections of the case vignette.

**** Modified Essay 4 has a total of 10 marks with 2 parts. ****

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist working in the community mental health clinic of a metropolitan hospital. You have been asked to review Julia, a 52-year-old woman who was referred by her general practitioner (GP). Julia is complaining of tearfulness, an inability to sleep, and excessive anxiety. Julia recently considered stopping her prescribed sertraline 100mg daily, because of weight gain and fatigue. The GP recommended a gradual reduction over a period of four weeks. Julia's current symptoms occurred in week 3 of a reducing regime, after she was reduced to a daily dose of 25mg. Her GP has diagnosed her with relapse of anxiety disorder but would like to get a second opinion.

You assess Julia as experiencing antidepressant withdrawal.

Question 4.2

Outline (list and justify) the steps you would take in managing Julia's presentation in the immediate term.

Please note: a list without justification will not be awarded any marks.

(4/10 marks)

Marking criteria:

A. (1 mark):

Advise Julia to return to a dose of 50 mg.

B. (1 mark):

Reassure Julia that her symptoms are likely to resolve over a few days.

C. (1 mark):

Once she is stabilised, she will need to reduce her sertraline in smaller amounts over a longer period.

D. (1 mark):

Write a letter to Julia's GP to outline a gradual tapering plan, monitoring of symptoms, and re-referral if further input is required.

E. (0 mark):

Recommend prescription of a liquid version of sertraline (available as a 'Special') to make the tapering more practical, as it will enable doses smaller than currently available tablet formulations.

RANZCP MEQ Examination Pilot Monday 24 November 2025

*** Modified Essay 5 has a total of 19 marks with 2 parts. ***

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist in the inpatient unit at a large teaching hospital in a metropolitan city. The report from a recent audit of prescribing patterns of hypnotics in the MH inpatient unit recommends that the mental health department develops a non-pharmacological strategy for improving night-time sleep in patients admitted to the inpatient unit.

Question 5.1

Describe (list and explain) how you would plan a strategy to implement the audit's recommendation.

Please note: a list without any explanation will not be awarded any marks.

(9/19 marks)

Marking criteria:

A. Engagement of stakeholders to evaluate the culture within the inpatient unit and to capitalise on all stakeholders' expertise and insight (5 marks):

- Nursing staff: Their insights are critical on patients' routines, and sleep disturbances; balancing competing responsibilities (e.g. night-time nursing observations and risk management vs protecting the continuity of patients' sleep; safety considerations to ensure safe access of patients to CPAP machine and other corded devices on the inpatient unit like white noise devices).
- Consumer consultant/Lived experience peer worker: Provide patients'/families'/carers' perspectives/feedback on sleep issues and individual and external factors interfering with sleep onset and maintenance on the inpatient unit (patient-centred approach).
- Pharmacist: Advice on medications interactions affecting sleep, identifying sleep-disrupting drugs, and timing of medications administration.
- Occupational therapist: Can advise on non-pharmacological interventions, assistive technologies, sensory modulation strategies, optimising sleep environments, running group programs on sleep hygiene education, relaxation techniques, activity scheduling, aromatherapy.
- Medical staff: To provide clinical insight and oversight.
- Hospital executive: Setting priorities, resource/funding allocation, staffing, short-term and long-term facility upgrades and modifications, investing in technologies, funding patient education programs, educational program for staff, quality assurance, outcome measurement tools, facilitating interdisciplinary collaboration (e.g. with sleep medicine and addiction departments), raising the needs at higher levels.
- Sleep psychologist/Sleep Clinic medical specialist: Opportunities and barriers to implement sleep hygiene and behavioural strategies, CBTi.

B. Literature review (1 mark):

- Looking for the evidence-base to support the strategy.
- Bench-marking against other inpatient units and best practices.

C. Re-auditing (1 mark):

- Evaluation as part of an audit cycle (i.e. establish baseline data on common sleep patterns and issues, and regular audits to assess the impact of the implementation of strategies against the agreed KPIs).
- Includes IT and data analysis.

D. Leadership (2 marks):

- Change management: develop a step-by-step plan for implementing changes.
- Documentation of the process, progress, staff involved, delays.
- Training and upskilling of staff on new protocols and interventions that will be implemented, face-to-face training of staff vs online modules on health service's intranet.
- Overseeing development of policy and procedure guidelines.
- Strategies to identify and respond to barriers to implementation (e.g. guidelines not being read, guideline being seen as too rigid, rigid practice culture on the inpatient unit, lack of training staff, high workload, competing priorities, guideline being seen as over-imposing and demanding).

RANZCP MEQ Examination Pilot Monday 24 November 2025

The information that is presented in italics in this question is a repetition of the earlier sections of the case vignette.

**** Modified Essay 5 has a total of 19 marks with 2 parts. ****

Each question within this modified essay question will be marked by a different examiner. The examiner marking one question will not have access to your answers to the other questions. Therefore, please ensure that you address each question separately and specifically. Answer this question fully, even if you believe that you have partly covered its content in your answers to other questions.

You are a junior consultant psychiatrist in the inpatient unit at a large teaching hospital in a metropolitan city. The report from a recent audit of prescribing patterns of hypnotics in the MH inpatient unit recommends that the mental health department develops a non-pharmacological strategy for improving night-time sleep in patients admitted to the inpatient unit.

Question 5.2

Describe (list and explain) non-pharmacological strategies you would consider at the inpatient unit level to improve night-time sleep for patients.

Please note: a list without explanation will not be awarded any marks.

(10/19 marks)

Marking criteria:

A. Environmental stimuli reduction (ward factors) (2 marks):

Refers to changing the environment to eliminate or minimise stimuli.

- Reduction of general ward noise (e.g. implementing quiet hours), reducing overnight staff activity, reducing overhead paging/alarms, appropriate visiting hours/duration.
- Environmental control and comfort including: lighting control (to mimic natural circadian rhythms), temperature and humidity control, comfortable and appropriate bedding.

B. Internal stimuli reduction (patient factors) (4 marks):

Refers to the patient's receptiveness to sleep.

- Can include relaxation and anxiety management strategies such as aromatherapy, massage, exposure to daylight to train up the circadian rhythm.
- Monitoring of caffeine intake, screen time, wind-down time, soothing pre-sleep activities (light reading, stretching), reducing nicotine intake.
- Teaching progressive muscle relaxation techniques, deep breathing.
- Promoting regular physical activity during the day.
- Managing mealtime schedules to avoid heavy meals before bedtime.
- Avoiding evening foods or drinks that might lead to gastroesophageal reflux during sleep in susceptible individuals.
- Avoiding evening physical exercise.
- Minimise exposure to common allergens like dust and pollen (i.e. closed windows, use of air purifiers, clean bedding and curtains) to avoid relapse of allergic or sensitivity reactions (e.g. nasal congestion) during sleep which can interrupt sleep.

C. Intermediate stimuli reduction (1 mark):

Refers to a barrier between environmental and internal stimuli reduction.

- Introduction of barriers such as ear plugs, eye masks/pillow, white noise devices, use of CPAP machine in OSA (obstructive sleep apnoea).
- Dampen the reception of environment stimuli by the internal system.

D. CBT-i (CBT for insomnia) (3 marks):

Individually or as a group activity. Components of CBT-I include:

- Sleep restriction therapy: Limiting time spent in bed to match actual sleep time, gradually increasing as sleep efficiency improves.
- Stimulus control therapy: Strengthening the association between the bedroom and sleep by limiting activities in bed to sleep and sex.
- Cognitive restructuring: Identifying and challenging maladaptive thoughts and beliefs about sleep.
- Sleep hygiene education: Teaching habits that promote good sleep, such as maintaining a cool, dark, quiet and non-humid sleep environment, avoiding alcohol, caffeine, nicotine, and heavy and spicy meals particularly in the evening/before bed.

- Relaxation training: For example, progressive muscle relaxation, deep breathing, and guided imagery to reduce arousal, anxiety and anxious ruminations and worries at bedtime.

Note to marker: Mentioning (CBT for insomnia) CB-i alone without any accompanying components will not accrue any marks.